

SCENE 6 — VT vs SVT: “Wide-Complex Showdown”

Scorecard

AV dissociation
cannon A waves
capture/fusion beats
QRS > 140ms
concordance
NW axis
prior MI/structural HD

VT

AV dissociation
cannon A waves
capture/fusion beats
QRS > 140ms
concordance
prior MI/structural HD

VT

MS > VT

VT Champ

VT CHAMP

ANNOUNCERS' BOARD

Unstable → cardioversion
never adenosine/verapamil

SVT Challenger

Stable monomorphic
→ procainamide
> amiodarone
> lidocaine

ARVC fighter

fibrofatty RV
epsilon wave

LBBB-VT

LBBB-VT

VT

VT

VT

ARVC fighter

fibrofatty RV

epsilon wave

LBBB-VT

LBBB-VT

VT

VT

VT

1. VT criteria scorecard

WHAT YOU SEE

Scorecard: AV dissociation, cannon A waves, capture/fusion beats, QRS>140, concordance, NW axis, prior MI/structural HD.

WHAT IT ENCODES

Features favoring VT in a wide-complex tachycardia: AV dissociation, cannon A waves, capture/fusion beats, very wide QRS (>140 ms RBBB / >160 LBBB), precordial concordance, northwest axis, and — most powerful — a history of structural heart disease/prior MI.

BOARD TRAP

WRONG: 'a regular wide-complex tachycardia in an older patient with prior MI is probably SVT with aberrancy.' Structural heart disease makes VT the default — treat as VT.

2. Stability does NOT rule out VT

WHAT YOU SEE

Top banner: 'stability does NOT rule out VT'.

WHAT IT ENCODES

Many VT patients are hemodynamically stable; tolerating the rhythm does not make it SVT. Diagnose by ECG criteria and history, not by how well the patient looks.

BOARD TRAP

WRONG: 'the patient is stable, so it must be SVT.' Hemodynamic stability does not distinguish VT from SVT.

3. VT champ vs SVT challenger

WHAT YOU SEE

Boxing ring: VT champion vs SVT challenger.

WHAT IT ENCODES

When in doubt about a wide-complex tachycardia, assume VT (it's more common and more dangerous, especially with structural heart disease). Misdiagnosing VT as SVT and giving AV-nodal blockers can be fatal.

BOARD TRAP

WRONG: 'default to SVT with aberrancy when unsure.' Default to VT — it's safer and usually correct.

4. Unstable → cardioversion; never adenosine/verapamil

WHAT YOU SEE

Announcers' board: unstable → cardioversion; never adenosine/verapamil.

WHAT IT ENCODES

An unstable wide-complex tachycardia gets immediate synchronized cardioversion. Do NOT give verapamil/diltiazem (or rely on adenosine) for undiagnosed wide-complex tachycardia — verapamil can cause hemodynamic collapse in VT.

BOARD TRAP

WRONG: 'try verapamil first for a wide-complex tachycardia.' In VT it can precipitate shock — cardiovert if unstable; use procainamide/amiodarone if stable.

5. ARVC fighter

WHAT YOU SEE

ARVC fighter: fibrofatty RV, epsilon wave, LBBB-morphology VT.

WHAT IT ENCODES

Arrhythmogenic RV cardiomyopathy causes VT with an LBBB morphology (RV origin) in a young patient; ECG shows epsilon waves and T-inversions in V1-V3, with fibrofatty RV replacement on imaging — a cause of exertional sudden death.

BOARD TRAP

WRONG: 'LBBB-morphology VT in a young athlete is benign RVOT VT.' Consider ARVC (structural, inherited, SCD risk) — not just idiopathic RVOT VT.

6. Stable monomorphic → procainamide > amiodarone > lidocaine

WHAT YOU SEE

Drug podium: procainamide, amiodarone, lidocaine.

WHAT IT ENCODES

For stable monomorphic VT, procainamide was superior to amiodarone in the PROCAMIO trial; amiodarone is a common alternative, and lidocaine is reserved largely for ischemic VT. Always have cardioversion ready.

BOARD TRAP

WRONG: 'amiodarone is clearly first-line for stable monomorphic VT.' Procainamide outperformed amiodarone (PROCAMIO) for termination with fewer adverse events.
